[redacted]

[redacted]

Review your new document for Illumination Foundation

From [redacted]

To: Illumination Foundation - [redacted]

Please conta[redacted]if you have any questions.

[redacted]

[redacted]

Aidin Patient Facesheet

Overview

Name: [redacted]
MRN: [redacted]
DOB: [redacted]
CSN: [redacted]

Gender: Male
Referral ID: [redacted]
Level of Care:
EDD:

Referral Info

Start of Service Date: [redacted]

Referral Source: [redacted]

Patient Info & Insurances

Admit Date: [redacted]

Attending Physician: [redacted]

Admit Reason:

Diagnosis / Procedure Code:

I60.9 - Nontraumatic subarachnoid hemorrhage, unspecified

M25.551 - Pain in right hip

S00.03XA - Contusion of scalp, initial encounter

S70.01XA - Contusion of right hip, initial encounter

S02.2XXA - Fracture of nasal bones, initial encounter for closed fracture

S02.32XA - Fracture of orbital floor, left side, init

S06.9X1D - Unspecified intracranial injury with loss of consciousness of 30 minutes or less, subsequent encounter

Facial Laceration - Facial Laceration

S02.85XA - Fracture of orbit, unspecified, initial encounter for closed fracture

Assault Victim - Assault Victim

Insurance Company: MEDI-CAL
Plan Name: [redacted]
Address:

Group Number: [redacted]
Policy Number: [redacted]
Effective Date: [redacted]

[redacted]

Expiration Date:

Group Name:

Insurance Company: MEDI-CAL

Plan Name: [redacted]

Address:

[redacted]

Group Name:

Group Number: [redacted]

Policy Number: [redacted]

Effective Date: [redacted]

Expiration Date:

Insurance Company: [redacted]

Plan Name:

Address:

[redacted]

Group Name: [redacted]

Group Number:

Policy Number: [redacted]

Effective Date: [redacted]

Expiration Date:

[redacted]

[redacted]
Requested Start of Service: No Date Set
Patient Location [redacted]

Documents

7 Attachments
Aidin Patient Facesheet
Facesheet | 15 minutes ago
Med List | 14 minutes ago
Vitals | 13 minutes ago
H_P | 13 minutes ago
Progress Note | 12 minutes ago
IF Referral Form [redacted]pdf | [redacted]nute ago

Notes

Cover Letter:

Pending updated PT/OT note. EDD [redacted]

Patient Info

Demographics

MRN:	[redacted]
Name:	[redacted]
Gender:	Male
DOB:	[redacted]
Age:	57
Height:	5' 9" ft
Weight:	2433.88 oz
SSN:	[redacted]
Allergies:	NO KNOWN ALLERGIES
Patient notes	

Contact Info

Patient/Caregiver Phone: [redacted]

Patient Address: [redacted]

Emergency Contact Info: No data available

Contact info notes

Care Details

Visit Number (CSN): [redacted]

Admission Date: [redacted]

Expected Discharge Date: No data available

Attending Physician: [redacted]

Patient Location: [redacted]

Patient Class: Inpatient

Financial Class: MEDI-CAL HMO

Service Line: AC Surgery

Readmission Risk: No data available

Referral Info

Diagnosis & Conditions

Diagnosis & Procedure Codes:
I60.9 (Nontraumatic subarachnoid hemorrhage, unspecified)
M25.551 (Pain in right hip)
S00.03XA (Contusion of scalp, initial encounter)
S70.01XA (Contusion of right hip, initial encounter)
S02.2XXA (Fracture of nasal bones, initial encounter for closed fracture)
S02.32XA (Fracture of orbital floor, left side, init)
S06.9X1D (Unspecified intracranial injury with loss of consciousness of 30 minutes or less, subsequent encounter)
Facial Laceration (Facial Laceration)
S02.85XA (Fracture of orbit, unspecified, initial encounter for closed fracture)
Assault Victim (Assault Victim)

Admit Reason: No data available

Insurance

[redacted]

Primary

Plan Name: [redacted]

Group Name:

Group Number: No data available

Policy Number: [redacted]

Effective Date: [redacted]

Expiration Date: No data available

Company Address: [redacted]

MEDI-CAL

Plan Name: MEDI-CAL / EDS

Group Name: No data available

Group Number: [redacted]

Policy Number: [redacted]

Effective Date: [redacted]

Expiration Date: No data available

Company Address: [redacted]

MEDI-CAL

Plan Name: MEDI-CAL / EDS

Group Name: No data available

Group Number: [redacted]

Policy Number: [redacted]

Effective Date: [redacted]

Expiration Date: No data available

Company Address: [redacted]

Insurance info notes

Followers

[redacted]

Add another location (view-only)

[redacted] M[redacted]

[redacted]

PCP

None

Patient Information

Patient Name	Gender Identity	Date of Birth	Age	Social Security Number
[redacted]	Male	[redacted]	57 y.o.	[redacted]

Address	Home Phone	Work Phone	Mobile Phone	Marital Status
[redacted]			[redacted]	Single

Insurance Information	Religion
[redacted]	Christian

Emergency Contacts

None on File

Other Contacts

None on File

Documents Filed to Patient

Power of Attorney	Living Will	Clinical Unknown	Study Attachment	Consent Form	ABN Waiver	After Visit Summary	Lab Result Scan	Code Status	MyChart Status
Not on File	Not on File	Not on File	Not on File	Not on File	Not on File	Filed	Not on File	Attempt Resu [Updated on [redacted]	Pending

Admission Information

Current Information

[redacted]	[redacted]	Admission Type	Admission Status
		Trauma	Confirmed Admission

Admission Date/Time	Discharge Date	Hospital Service	Auth/Cert Status
[redacted]		Acute Care Surgery	Incomplete

Hospital Area	Unit	Room/Bed
[redacted]	[redacted]	

Hospital Account

[redacted]

[redacted]

Name	Acct ID	Class	Status	Primary Coverage
[redacted]	[redacted]	Inpatient	Open	[redacted]

Guarantor Account (for Hospital Accou[redacted]

Name	Relation to Pt	Service Area	Active?	Acct Type
[redacted]	Self	[redacted]	Yes	Personal/Family

Address	Phone
[redacted]	

Coverage Information (for Hospital Ac[redacted]

F/O Payor/Plan	Precert #
[redacted]	

Subscriber	Subscriber #
[redacted]	[redacted]

Address	Phone
[redacted]	

[redacted]

[redacted] (57 y.o. M) [redacted]

[redacted]

Scheduled Meds Sorted by Name

[redacted]

Legend:

Given	Hold	Not Given	Due	Canceled Entry	Other Actions
Time	Time	(Time)	Time	Time	**Time-Action**

Inactive: ✕ | Active | Linked: ∞

Medications

[redacted]

acetaminophen (TYLENOL) tablet 650 mg Dose: 650 mg Freq: Every 6 hours Route: Oral [redacted] Admin Instructions: Adults & Peds 12 years and older: 4,000 mg acetaminophen/24 hrs in divided doses; Infants, Children, & Adolescents: [redacted] mg/kg oral and up to 20 mg/kg rectal every [redacted] hours; Do NOT exceed 5 doses in 24 hours; Max Daily Dose = 75 mg/kg/day not to exceed 1,000 mg single dose or 4,000 mg/day	[redacted]	0500 1100 1700 2300	0500 1100 1700 2300	0500 1100 1700 2300	0500 1100 1700 2300	0500 1100 1700 2300	0500 1100 1700 2300	0500 1100 1700 2300
enoxaparin (LOVENOX) syringe 30 mg Dose: 30 mg Freq: 2 times daily Route: SubQ [redacted] Admin Instructions: Spinal or epidural hematomas, and paralysis, [redacted] occur with epidural or spinal anesthesia or spinal puncture in patients anticoagulated with LMWH, heparinoids, or fondaparinux.	[redacted] 2100	0900 2100	0900 2100	0900 2100	0900 2100	0900 2100	0900 2100
folic acid (FOLVITE) tablet 1 mg Dose: 1 mg Freq: Daily Route: Oral [redacted]	[redacted]	0900	0900	0900	0900	0900	0900
levETIRAcetam (KEPPRA) tablet 1,000 mg Dose: 1,000 mg Freq: 2 times daily Route: Oral [redacted]	[redacted] 2100	0900 2100	0900 2100	0900 2100	0900 **2059-D/C'd**	✕	∞
melatonin tablet 3 mg Dose: 3 mg Freq: Every evening at bedtime Route: Oral [redacted]	2100	2100	2100	2100	2100	2100	2100
multivitamin (THERAGRAN) 1 tablet Dose: 1 tablet Freq: Daily Route: Oral [redacted]	[redacted]	0900	0900	0900	0900	0900	0900

[redacted]

[redacted]

polyethylene glycol (MiRALAX) packet 17 g [redacted] Dose: 17 g Freq: Daily Route: Oral [redacted] Admin Instructions: Dissolve powder in 120 - 240 mL of clear liquid and administer immediately.		0900	0900	0900	0900	0900	0900
senna-docusate (PERICOLACE) [redacted] mg 1 tablet [redacted] Dose: 1 tablet Freq: 2 times daily Route: Oral [redacted]	2100	0900 2100	0900 2100	0900 2100	0900 2100	0900 2100	0900 2100
tamsulosin (FLOMAX) 0.4 mg capsule 0.4 mg [redacted] Dose: 0.4 mg Freq: Daily Route: Oral [redacted] Admin Instructions: Do not crush/chew. Nurse to do patient food-drug interaction education: Avoid grapefruit juice. (May increase tamsulosin concentration)		0900	0900	0900	0900	0900	0900
thiamine tablet 100 mg [redacted] Dose: 100 mg Freq: Daily Route: Oral [redacted]		0900	0900	0900	0900	0900	0900
Medications [redacted]							

Continuous Meds Sorted by Name [redacted]

Legend:

Given	Hold	Not Given	Due	Canceled Entry	Other Actions
Time	Time	(Time)	Time	Time	**Time-Action**

Inactive: X | Active | Linked: ∞

Medications [redacted]

PRN Meds Sorted by Name [redacted]

Legend:

Given	Hold	Not Given	Due	Canceled Entry	Other Actions
Time	Time	(Time)	Time	Time	**Time-Action**

Inactive: X | Active | Linked: ∞

Medications [redacted]
artificial tears ophthalmic solution 1 drop Dose: 1 drop Freq: Every 2 hour PRN Route: EACH EYE PRN Reason: Dry Eyes [redacted] Admin Instructions: Equivalent to Artificial Tears

[redacted]

dextrose injection 12.5 g Dose: 12.5 g Freq: As needed Route: IV PRN Comment: See Admin Instructions [redacted] Admin Instructions: -BG below 50 mg/dL and patient not alert, NPO, or on TPN: 50 mL (25 grams) of IV D50%. -BG 50-69 mg/dL and patient not alert, NPO, or on TPN: 25 mL (12.5 grams) of IV D50%.							
Or **dextrose injection 25 g** Dose: 25 g Freq: As needed Route: IV PRN Comment: See Admin Instructions [redacted] Admin Instructions: -BG below 50 mg/dL and patient not alert, NPO, or on TPN: 50 mL (25 grams) of IV D50%. -BG 50-69 mg/dL and patient not alert, NPO, or on TPN: 25 mL (12.5 grams) of IV D50%.							
fentaNYL (SUBLIMAZE) 50 mcg/mL injection Freq: Code/Trauma/Procedure Medication Route: IV [redacted]	✂	✂	✂	✂	✂	✂	✂
insulin lispro (HumaLOG, ADMELOG) injection [redacted] Units Dose: [redacted] Units Freq: Before Meals and at Bedtime as needed Route: SubQ PRN Reason: High Blood Sugar PRN Comment: correction scale [redacted] Admin Instructions: BG <70 = hold rapid-acting insulin and see hypoglycemia measures [redacted] = No correction [redacted] = 2 units [redacted] = 4 units 251-300 = 6 units [redacted] = 8 units >350 = 10 units and call provider on-call RCRA waste							
iopamidol (ISOVUE-370) 76 % solution 100 mL Dose: 100 mL Freq: Once as needed Route: IV PRN Reason: Contrast [redacted]	✂	✂	✂	✂	✂	✂	✂
sodium chloride [redacted] (NS) IV solution [redacted] Admin Instructions: [redacted]	✂	✂	✂	✂	✂	✂	✂
Medications	[redacted]						

[redacted]

[redacted]

Vital Sign Min/Max (last 24 hours)

Value	Min	Max
Temp	[redacted]°F [redacted]°C)	[redacted]°F [redacted]°C)
Heart Rate	71	100 !
Respirations	16	17
Blood Pressure: Systolic	96 !	120
Blood Pressure: Diastolic	67	77
SpO2	97 %	99 %

Vitals (last day)

Date/Time	Temp	Temp src	Pulse	Resp	BP	SpO2	Weight	Arterial Line BP	MAP	Who
[redacted]	[redacted]°F [redacted]°C)	Oral	90 !	17	96/67 !	99 %	—	—	—	[redacted]
	[redacted]°C)	Oral	79	17	106/71 !	98 %	—	—	—	
	—	—	—	—	—	—	69 kg (152 lb [redacted]oz)	—	—	
	[redacted]°C)	Oral	84	17	[redacted] !	97 %	—	—	—	
	[redacted]°C)	Oral	100 !	16	98/71 !	97 %	—	—	—	
	98 °F [redacted]°C)	Oral	71	16	101/67 !	98 %	—	—	—	
	[redacted]°F [redacted]°C)	Oral	93 !	16	120/77	99 %	—	—	—	
	[redacted]°C)	Oral	72	16	100/68 !	96 %	—	—	—	
	[redacted]°C)	Oral	94 !	18	108/73 !	98 %	[redacted]kg (153 lb [redacted]oz)	—	—	

[redacted]

[redacted]

[redacted] H&P Date of Service [redacted]
Physician Signed
Acute Care Surgery

ADULT TRAUMA H&P / CONSULT

ADULT TRAUMA Level B

MODE OF TRANSPORTATION: ambulance

TRANSFERRING FACILITY/PHYSICIAN: Not Applicable

Team Arrival

	Trauma Team Members	Time Called	Time Arrived
Attending	[redacted]	19:27	19::27
Sr		"	"
[redacted]		"	"

HISTORY

GENDER: Male

TIME OF INJURY: –

MECHANISM OF INJURY: Assault? Bike fall

LOSS OF CONSCIOUSNESS: No

RESPIRATORY ARREST: No
INTUBATED: no

PREHOSPITAL VITAL SIGNS: -

PREHOSPITAL GCS: 15 in the field

HEMODYNAMIC INSTABILITY: NO

SUSPICION OF ABUSE (DOMESTIC ABUSE, ELDER ABUSE, CHILD ABUSE): NO

CERVICAL COLLAR ON ARRIVAL TO ED: NO

BACKBOARD ON ARRIVAL TO ED: NO

TXA ADMINISTERED EN ROUTE: NO

PATIENT COMPLAINT: Face pain

DETAILS OF INCIDENT: 57 y.o. male with no pertinent history presents to the ED via EMS with head injuries.

Patient reports he was riding his bike when he fell onto the side walk and had a head strike. Patient endorses [redacted] head and face pain. Denies loss of consciousness.

[redacted]

Per EMS, they were called out because patient was found outside of a store and there was concern for possible assault after bystander noted multiple injuries to the head. Patient denies assault.

Denies blood thinners.

Pt. Denies assault, but multi lacs and bruising to left eye and face.

GLASCOW COMA SCORE FOR ADULT

EYES: Spontaneous: 4

VERBAL: Oriented, converses normally: 5

MOTOR: Obeys: 6

IMMUNIZATION

TETANUS VACCINE UP TO DATE?: Unknown

Administered in Trauma Bay: NO

PAST MEDICAL HISTORY

Medical History
Past Medical History:

Diagnosis	Date

- Asthma

- **Chronic ongoing medical problems:** Asthma
- **Resolved medical problems:** None

Family / acquaintance available: None

SURGICAL HISTORY

Surgical History
History reviewed. No pertinent surgical history.

FAMILY HISTORY

noncontributory

MEDICATIONS

Medications:

Medications Ordered Prior to Encounter
No current facility-administered medications on file prior to encounter.

No current outpatient medications on file prior to encounter.

ALLERGIES

Allergies
No Known Allergies

SOCIAL HISTORY

Social History

[redacted]onic History

[redacted] status: Not on file

Spouse name: [redacted]

- Number of children: Not on file
- Years of education: Not on file
- Highest education level: Not on file

Occupational History

- Not on file

Tobacco Use

- Smoking status: Not on file
- Smokeless tobacco: Not on file

Substance and Sexual Activity

- Alcohol use: Not on file
- Drug use: Not on file
- Sexual activity: Not on file

Other Topics Concern

- Not on file

Social History Narrative

- Not on file

Social Drivers of Health

Tobacco Use: Not on file
Alcohol Use: Not on file
Financial Resource Strain: Not on file
Food Insecurity: Not on file
Transportation Needs: Not on file
Physical Activity: Not on file
Stress: Not on file
Social Connections: Not on file
Intimate Partner Violence: Not on file
Depression: Not At Ri[redacted]
PHQ-2
- PHQ-2 Score: 0

Housing Stability: Not on file
Utilities: Not on file

Tobacco: -
Alcohol: Yes
Drug Use: None

PHYSICAL EXAM

Initial Vital Signs:
Temp: [redacted] °F (37 °C)
Heart Rate: (I) 89
Respirations: 18
Blood Pressure: (I) 115/88
SpO2: 99 %
MAP (mmHg): 106 mmHg
Pain Score: 9
Body mass index is [redacted] kg/m².

Rhythm on monitor: Normal sinus rhythm

Head: normocephalic / atraumatic

Eyes: Pupils 3 mm, bilaterally reactive to light or Extraocular muscles movement intact

ENT: oropharynx clear, **Small laceration to bilateral maxillary area. Dried blood present on face and**

[redacted]

occiput. Dried blood in nares. Internal mouth atraumatic, no blood.
Left eye: Hemorrhage present. **Periorbital ecchymosis and swelling to left eye.**

Neck: In a C-collar, Supple, non-tender, and Trachea midline

Respiratory/Chest: breath sounds clear & equal, no retractions, no deformity, no tenderness, and no crepitance

Cardiovascular: regular rate and rhythm and no murmurs auscultated

Abdomen: soft, non-tender, non-distended, normal bowel sounds

Back: No tenderness across thoracic / lumbar spine and No tenderness to palpation

Pelvis: non-tender, stable to anterior-posterior/lateral compression

Right Upper Extremity: no palpable long bone deformities
- Motor: able to move extremity and strength normal
- Sensation: Normal / intact

Left Upper Extremity: no palpable long bone deformities
- Motor: able to move extremity and strength normal
- Sensation: Normal / intact

Right Lower Extremity: no palpable long bone deformities, **Right hip tenderness.**
- Motor: able to move extremity and strength normal
- Sensation: Normal / intact

Left Lower Extremity: no palpable long bone deformities
- Motor: able to move extremity and strength normal
- Sensation: Normal / intact

Rectal: Good gluteal squeeze

Genitourinary: normal genitalia

Skin: grossly intact

Pulses Exam

Right	Left
Carotid: Palpable (2+)	Carotid: Palpable (2+)
Radial: Palpable (2+)	Radial: Palpable (2+)
Fermoral: Palpable (2+)	Fermoral: Palpable (2+)
Dorsalis Pedis: Palpable (2+)	Dorsalis Pedis: Palpable (2+)
Posterior Tibial: Palpable (2+)	Posterior Tibial: Palpable (2+)

Adult Trauma Phys Exam Graphical Documentation

LABS

Recent Labs

Lab	[redacted]	
HGBPOC	[redacted]	

[redacted]

Recent Labs

Lab	[redacted]
WBC	[redacted]
HGB	
HCT	
MCV	
MCH	
MCHC	
RDW	
PLT	73*
MPV	[redacted]

Glucose (POC)

Date	Value	Ref Range	Status
[redacted]	89	70 - 140 mg/dL	Final

Comment:
[redacted]
Result Not Confirmed
BLTS:Venous
Normal Range for Glucose (POC) had been changed as [redacted]

Glu, Random

Date	Value	Ref Range	Status
[redacted]	90	70 - 140 mg/dL	Final

Comment:
Random glucose reference range: [redacted] *mg/dL*
Fasting glucose reference range: [redacted] *mg/dL*

Venous blood sugar:

Recent Labs

Lab	[redacted]	
GLUCOSE	89	90

Recent Labs

Lab	[redacted]
NA	137
K	[redacted]
CL	102
CO2	21*
BUN	9
CREATININE	[redacted]
TOTALBILI	
AST	22
ALT	18
ALBUMIN	[redacted]
ALKPHOS	80
ANIONGAP	14
CA	[redacted]

[redacted]

Recent Labs

Lab	[redacted]
LACTATE	[redacted]

No results for input(s): "COLORU", "CLARITYU", "SPECGRAV", "PHUR", "LEUKUR", "NITRITE", "PROTEIN", "GLUCOSEUA", "KETONESU", "UROBILI", "BILI", "BLOODUA", "HYALINE", "GRANCASTS", "FATTYCASTS", "WAXYCASTS", "RBCCASTS", "WBCCASTS", "MUCUS", "WBCUA", "RBCUA", "SQUAMEPI", "BACTERIA", "AMORPH" in the last 72 hours.

Drug Screen:
No results for input(s): "LABAMPH", "LABBARB", "LABBENZ", "LABCANNAB", "LABCOCAINE", "LABOPIA" in the last 72 hours.

Invalid input(s): "LABPCP", "LABDAUPH"

ETOH level:

Recent Labs

[redacted]	
LABETH	0.026*

IMAGING

CT Head WO Contrast
Result Date [redacted]
EXAMINATION: CT HEAD WO CONTRAST [redacted] HISTORY: 57 year old Male undergoing interval evaluation of the intracranial hemorrhage. COMPARISON: [redacted] EDURE: Utilizing the GE CT scanner, contiguous scans of the head were obtained. Image data were processed using Standard and Boneplus algorithms. Sagittal and coronal reconstructions were provided. CT DOSE: The maximum CTDIvol = 61 mGy. Total DLP = 2303 mGy-cm FINDINGS: White matter/Parenchyma: Similar small foci of subarachnoid hemorrhage involving the bilateral frontal lobes no significant regional mass effect. Sulci: Mild sulcal prominence. Ventricles: Mild ventricular prominence. Extra-axial spaces: Unremarkable. Aerated spaces: Unchanged. Skull and visible facial bones: Unchanged multiple facial fractures, better seen on recent maxillo [redacted] Similar periodontal disease. Partially visualized cervical fusion hardware. Other: Similar facial and scalp edema/hematoma, with scattered subcutaneous emphysema. Intracranial atherosclerosis. IMPRESSION: ATTN: Resident Preliminary Report Not Yet Reviewed b [redacted] M Stable small volume subarachnoid hemorrhage. No new hemorrhage or significant mass effect. No significant interval change since [redacted]

CT Head WO Contrast
Result Date [redacted]
EXAMINATION: CT HEAD WO CONTRAST [redacted] PM HISTORY: 57 years old Male with AMS, repeat for SAH. COMPARISON: CT trauma head [redacted]
PROCEDURE: Utilizing the GE CT scanner, contiguous scans of the head were obtained. Image data were processed using Standard and Boneplus algorithms. Sagittal and coronal reconstructions were provided. CT Dose: CTDIvol 62 mGy. DLP 1152 mGy-cm FINDINGS: White matter/Parenchyma: Several small bilateral foci of subarachnoid hemorrhage, stable from comparison. Sulci: Unremarkable. Ventricles: Unremarkable. Extra-axial spaces: Unremarkable. Aerated spaces: No significant change. Skull and visible facial bones: Multiple facial fractures described on recent maxillofacial CT. Other: None. IMPRESSION: Stable small volume bilateral subarachnoid hemorrhage. No new or expanding hemorrhage. Some hemorrhagic foci again seem to be subcortical, raising the possibility of DAI in the appropriate clinical setting. [redacted] Reviewed Images Personally and Agree With Interpretation, as currently edited. Electronically SIGNED [redacted]

CT Trauma Head/C Spine WO Contrast
Result Date [redacted]
EXAMINATION: CT TRAUMA HEAD/C SPINE WO CONTRAST [redacted] HISTORY: 57 year old Male with assault. COMPARISON: None PROCEDURE: Utilizing the GE CT scanner, contiguous axial scans of the head and cervical spine were obtained. Image data were processed using Standard and Bone algorithms. 2D thin section coronal and sagittal reformatted images were created. CT DOSE: CTDIvol 62 mGy. DLP 1254 mGy-cm FINDINGS: HEAD: Fracture: Please see concurrent maxillofacial CT Intracranial hemorrhage: Several small bilateral foci of subarachnoid hemorrhage. Soft tissues: Extensive left facial and periorbital soft tissue swelling with soft tissue enhancement and subcutaneous emphysema. Sulci: Normal. Ventricles: Normal. White matter/parenchyma: Normal. Aerated spaces: Partial fluid opacification of the bilateral maxillary and frontal sinuses and ethmoid air cells. Other: None. CERVICAL SPINE: Fracture: No acute fractures. Type 2 dens fracture with intact fixation screw and regional heterotopic ossification. 1 to 2 mm [redacted] of the fracture fragments and 2 mm perihardware lucency, best seen on sagittal series 602, image 71. Alignment: Reversal cervical lordosis. Spinal stenosis: None significant. Mild to moderate degenerative changes of the cervical spine from C4-C7. Neural foraminal narrowing: Moderate left and mild right neural foraminal stenosis at C5-6. Mild left neural foraminal stenosis at C6-7. Paraspinal soft tissues: Normal. Other: Biapical pleural scarring IMPRESSION: Several small foci of hyperdense bifrontal subarachnoid hemorrhage. Some hemorrhagic foci seem to be subcortical, raising the possibility of DAI in the appropriate clinical setting. No clear evidence of acute cervical spine fracture or subluxation. Chronic-appearing type II dens fracture with fixation screw. Perihardware lucency measuring up to 2 mm raises the possibility of screw loosening and/or infection. Recommend comparison with prior imaging. Multiple facial fractures described on concurrent maxillofacial CT. Multiple dental caries, missing teeth and periapical inflammatory lesions. DIC [redacted] Reviewed Images Personally and Agree With Interpretation, as currently edited. Electronically SIGNED / Radi [redacted]

XR Chest Single View
Result Date [redacted]
XR CHEST SINGLE VIEW [redacted] HISTORY: 57 years -old Male being evaluated for Trauma. COMPARISON: None. TECHNIQUE: AP view of the chest was obtained. FINDINGS: The cardiac silhouette is within normal limits. The mediastinal contour is within normal limits. Pulmonary vasculature is within normal limits. There are bibasilar streaky opacities. Otherwise, lungs are well aerated without focal consolidation or mass lesion. There is no pleural effusion present. There is no pneumothorax. Scattered osseous degenerative changes are visualized. There are left lower chronic/old rib fracture deformities with suggestion of cortical break and lucency. The visualized soft tissues appear within normal limits. IMPRESSION: Chronic/old left lower rib fracture deformities with acute component not excluded. Bibasilar atelectasis. DICTATE [redacted] I Reviewed Images Personally and Agree With Interpretation, as currently edited. Electronically SIGNED / Radiolog [redacted]

XR Pelvis Ap Only
Result Date [redacted]
XR PELVIS AP ONLY [redacted] PM HISTORY:57 years old Male with trauma TECHNICAL DATA: Frontal view was obtained of the pelvis. COMPARISON: No prior FINDINGS: Right total hip replacement is in good position. There is severe narrowing of the left hip joint with flattening of the femoral head with areas of patchy sclerosis. [redacted] represent AVN. There is no acute fracture or dislocation. Right hip soft tissue swelling is noted. IMPRESSION: No acute fracture or dislocation. DICTATED [redacted] Reviewed Images Personally and Agree With Interpretation, as currently edited. Electronically SIGNED / Radiologist [redacted]

CT Maxillofacial WO Contrast
Result Date [redacted]
EXAMINATION: CT MAXILLOFACIAL WO CONTRAST [redacted] PM HISTORY: 57 years old Male with assault, c/f L periorbital fractures. COMPARISON: None PROCEDURE: Using the GE CT scanner, contiguous noncontrast scans were obtained from above the orbital rims to below the mandible. Image data were processed using Standard and Boneplus algorithms. Coronal and sagittal reformatted images were then generated. FINDINGS: There are mildly displaced comminuted bilateral nasal bone fractures extending posteriorly into the ethmoid air cells. There is a minimally displaced left orbital floor fracture, and a nondisplaced fracture of the left inferolateral orbital rim extending laterally and posteriorly to involve the zygomatic arch. There is a mildly displaced and overriding right superior orbital wall fracture. There is no evidence of intraorbital fat or extraocular muscle herniation/entrapment. There are small

[redacted]

bilateral extraconal hematomas. Otherwise, the orbital contents appear normal. There is extensive left facial/malar and left periorbital soft tissue swelling with scattered subcutaneous emphysema and regions of hyperdense hemorrhage. There is near complete fluid opacification of the right frontal sinus and mild to moderate fluid opacification of the left frontal sinus, ethmoid air cells, and bilateral maxillary sinuses. The mastoid air cells and middle ear cavities are normally aerated. Please see concurrent CT trauma head/C-spine report for intracranial and cervical findings. IMPRESSION: Mildly displaced comminuted bilateral nasal bone fractures extending posteriorly into the ethmoid air cells. Minimally displaced left orbital floor fracture without evidence of intraorbital fat or extraocular muscle herniation/entrapment. Nondisplaced left inferolateral orbital rim fracture extending along the zygomatic arch. Small extraconal hematoma. Mildly displaced and overriding right superior orbital wall fracture. Small extraconal hematoma. Extensive left facial/malar and left periorbital soft tissue swelling, hematoma, and scattered subcutaneous emphysema. Mild to moderate fluid opacification of the frontal ethmoid, and maxillary sinuses, a component of which [redacted] represent small volume hemorrhagic products (in the ethmoid air cells and left maxillary sinus). CT DOSE: CTDIvol 62 mGy. DLP 1254 mGy-cm DICTATED [redacted] wed Images Personally and Agree With Interpretation, as currently edited. Electronically SIGNED / Radi[redacted]

Other Imaging Results: (Reads from Outside Hospitals):

None

Outside images reviewed w/ Radiology: NO

BCVI & Aspirin Considerations

Does this patient have a blunt cerebral vascular injury (BCVI)? No

PROCEDURES

Foley: No

Nasogastric Tube: No

Chest Tube(s): No

Central Venous Catheter: No

Common femoral arterial access obtained? No
pREBOA deployed? No

Trauma Blood Products Administered: None

Other procedure: N/A

ASSESSMENT & PLAN

Is this an isolated neurotrauma patient for which admission is being considered? No

Is this patient frail, greater than the age of 60 or do they have multiple comorbidities? No

Traumatic Injuries:
External/Skin: No injuries
Neck (excluding spine): No injuries
Abdomen/Pelvis Contents: No injuries
Extremities/Bony Pelvis: No injuries
Head/Face:
possible DAI (Several small foci of hyperdense bifrontal subarachnoid hemorrhage.)
SAH

Mildly displaced comminuted bilateral nasal bone fractures extending posteriorly into the ethmoid air cells.
Minimally displaced left orbital floor fracture without evidence of intraorbital fat
extraocular muscle herniation/entrapment.
Nondisplaced left inferolateral orbital rim fracture extending along the zygomatic arch.
Small extraconal hematoma
Mildly displaced and overriding right superior orbital wall fracture
Small extraconal hematoma.
Extensive left facial/malar and left periorbital soft tissue swelling, hematoma, and scattered subcutaneous emphysema.
small volume hemorrhagic products (in the ethmoid air cells and left maxillary sinus).

Thorax: No injuries
Spine: No injuries
Non-Trauma Diagnoses/Chronic Ongoing Medical Problems:
Chronic-appearing type II dens fracture with fixation screw
Perihardware lucency measuring up to 2 mm raises the possibility of screw loosening and/or infection.
Multiple dental caries,
missing teeth and periapical inflammatory lesions.
Chronic/old left lower rib fracture deformities

Multiple System Trauma Patient
Acute pain
- Multimodal pain control including parenteral controlled substances.

Altered Mental Status
TBI
Encephalopathy - *traumatic brain injury, alcohol*
GCS @ Admission Glasgow Coma Scale Score: 15
Most Recent Documented **GCS**: Glasgow Coma Scale Score: 14
- **precautions:** minimize benzos, anticholinergics, and opioids, reduce intrinsic deliriogenic factors, Maintain nutrition. Reduce extrinsic deliriogenic factors: including sleep-wake cycles, sleep hygiene, reassurance and re-orientation, try to avoid restraints.

Alcohol Abuse
- BAIT consult

Lab Results

Component	Value	Date/Time
LABETH	0.026 (H)	[redacted]

Coagulopathy
thrombocytopenia
Recent Labs

Lab	[redacted]
PLT	73*

Incidental Findings:
None

SDOH:
#None

Plan:
- Pan Scan plus Max Face (done)
- Head CT
  - Repeat Heat CT at 6hrs (done)
- Pending UDS
- UA

[redacted]

- Labs (done)
- Frequent Neuro checks
- TXA
- Kepra
- OMFS consult
    - Ophthalmology consult for evaluation of L orbital trauma
    - Apply bacitracin to abrasions TID
    - Oral hygiene cares
    - **Sinus Precautions**
        - It is important that you avoid all of the following:
            - Blowing your nose - It is best to wipe away nasal secretions carefully. After 2 weeks, if you must blow your nose, blow gently through both sides at the same time. Do not pinch your nose; do not blow just one side at a time.
            - Sneezing - If you must sneeze, keep your mouth open and do not pinch your nose closed.
            - Sucking - Do not drink through a straw. Do not smoke.
            - Blowing - Do not play a wind instrument. Do not blow up balloons.
            - Pushing or lifting - Do not lift or push objects weighing more than 20 pounds.
            - Bending over - Keep your head above the level of your heart. Sleep with your head slightly raised
- Ophtalmology consult
    - - No acute intervention per ophthalmology
    - - Defer further management of orbital fracture repair to facial trauma service
    - - Start ice to periorbital swelling for 20 min at a time every [redacted] hours for first 48 hours
    - - No nose blowing
    - - Patient should have outpatient ophthalmology follow up, on call resident to arrange.
    - - Review RD signs and symptoms with patient: [redacted] to ER for flashing lights, shower of floaters, or dark veil over vision
    - - RD precautions: no heavy lifting, straining, or bending over for 1 week
- NSGY consult
    - **Recommendations**:
        - No emergent neurosurgical intervention
        - HOB >30 degrees
        - PLT >100, INR [redacted]
        - Continue to monitor until pt no longer intoxicated from EtOH
        - Other traumatic injuries per ACS, Optho, Facial trauma
    - - Pt wil require outpt f/u w/ NSGY (ED DC Clinic) in 4 weeks
    - -Pt will require CTH prior to f/u
    - -Clinic scheduler contacted in regards to scheduling appointment
    - -NSGY to follow on a PRN basis, please reach out to NSGY if any questions/concerns arise regarding the neurosurgical care of this patient.

CONSULTS:
VIP
BAIT
Neurosurgery
OMFS (Facial trauma)
Ophtalmology (Left eye)

Case discussed with attending trauma physician [redacted]

Dispo: per ED

[redacted]

ATTENDING ATTESTATION:
In this encounter on the above listed date of service, this critically ill patient received my full attention for a **total time of 35 minutes** providing critical care at the immediate bedside, in the emergency room, or on the unit, although this time was not necessarily continuous. The time involved in the performance of

[redacted]

[redacted] billable procedures was not counted toward critical care time.
This patient is critically ill with the following critical care-related diagnoses
Critical Care-Related Diagnoses:
Principal Problem:
SAH (subarachnoid hemorrhage) (HCC-CMS)
Active Problems:
Hip pain, acute, right
I provided the following critical care treatments:
Critical Care Treatment(s) Provided:
- Frequent neurological status monitoring
- Seizure Precautions and Appropriate Interventions

I was physically present during the evaluation and care of the patient. I performed the key and critical parts of service, discussed with the resident, personally performed critical parts of the examination, as well as participated in the management of the patient. I have reviewed the resident's note and agree with the documented findings and plan of care.

[redacted]

Revision History

Date/Time	User	Provider Type	Action
[redacted]	[redacted]	Physician	Sign
		Resident	Sign

[redacted]

[redacted]

[redacted] Progress Notes Date of Service [redacted]
Resident
Acute Care Surgery
Cosign Needed

ACS note

Primary Diagnosis:

Chief Complaint
Patient presents with

- Facial Laceration
  Multi to face
- Assault Victim
  Pt. Denies assault, but multi lacs and bruising to left eye.

HPI at Admissi [redacted] 7 y.o. male admitted o [redacted] Patient has a past medical history of Asthma.

57 y.o. male with no pertinent history presents to the ED via EMS with head injuries.

Patient reports he was riding his bike when he fell onto the side walk and had a head strike. Patient endorses [redacted] head and face pain. Denies loss of consciousness.

Per EMS, they were called out because patient was found outside of a store and there was concern for possible assault after bystander noted multiple injuries to the head. Patient denies assault.

Interval:

[redacted] No acute events overnight, able to have BM and void
[redacted] No acute events overnight, able to void and have BM, afebrile, VSS, pending PT/OT and CM

Surgical HistoryMedical History
History reviewed. No pertinent surgical history.

Past Medical History:

Diagnosis	Date

- Asthma

Objective:

Vitals Temp: [redacted] °F [redacted] °F [redacted] °C)] [redacted] °F [redacted] °C)
Heart Rate: [redacted] 72
Respirations: [redacted] 16
Blood Pressure: [redacted] 100/68
SpO2: [96 %-100 %] 96 %
[redacted]
In: 120 [P.O.:120]
Out: 975 [Urine:975]
Net: -855

Physical Exam:

Constitutional: BP (!) 100/68 (BP Location: Left Upper Arm, BP Patient Position: Semi-Fowler) | Pulse 72 | Temp [redacted] °F [redacted] °C) (Oral) | Resp 16 | Ht 1.753 m (5' 9") | Wt [redacted] kg (153 lb [redacted] oz) | SpO2 96% | BMI [redacted] kg/m², Body mass index is [redacted] kg/m².
GCS @ Admission Glasgow Coma Scale Score: 15
Most Recent Documented **GCS**: Glasgow Coma Scale Score: 15
Respiratory: SpO2: 96 % [redacted] Respirations: 16 [redacted]
Chest/Cardiac: Heart Rate: 72 [redacted] Pressure: 100/68 [redacted]
Pertinent findings: facial laceration, bilateral periorbital edema and ecchymosis, no diplopia, abdomen soft, non-tender and non-distended

[redacted]

Lab/Studies:

Vitals: [redacted]

BP: (!) 100/68
Pulse: 72
Resp: 16
Temp: [redacted] °F [redacted] °C)
SpO2: 96%

Recent Labs

Lab	[redacted]									
HGB	–	13.5	--	–	–	--	–	–	[redacted]	[redacted]
PLT	–	23*	--	–	–	--	–	–	443*	460*
WBC	–	[redacted]	--	–	–	--	–	–	[redacted]	[redacted]
LACTATE	–	[redacted]	--	–	–	--	–	–	--	–
MG	–	–	--	–	–	--	–	[redacted]	--	[redacted]
PHOS	–	–	--	–	–	--	–	[redacted]	--	1.4
NA	–	137	--	–	–	--	–	136	--	138
K	–	[redacted]	--	–	–	--	–	4.5	--	4.6
BUN	–	9	--	–	–	--	–	7	--	10
CREATININE	–	[redacted]	--	–	–	--	–	[redacted]	--	[redacted]
TOTALBILI	–	[redacted]	--	–	–	--	–	–	--	--
AST	–	22	--	–	–	--	–	–	--	–
ALT	–	18	--	–	–	--	–	–	--	–
ALKPHOS	–	80	--	–	–	--	–	–	--	–
GLUCOSE	89	90	95	118	100	95	102	84	--	102

Recent Labs

Lab	[redacted]
COLORU	Yellow
CLARITYU	Clear
SPECGRAV	1.016
PHUR	[redacted]
LEUKUR	Negative
NITRITE	Negative
PROTEIN	Trace*
GLUCOSEUA	Negative
KETONESU	Trace*
UROBILI	[redacted]
BILI	Negative
BLOODUA	Moderate*
MUCUS	Present
WBCUA	1
RBCUA	None Seen
BACTERIA	None Seen

Lab Results

Component	Value	Date/Time
LABETH	0.026 (H)	[redacted]
LABCANNAB	Pos unconfirmed (A)	
AMPHETAMINE	Pos unconfirmed (A)	
FENTANYL	Pos unconfirmed (A)	
LABOPIA	Negative	
LABCOCAINE	Negative	
PCP	Pos unconfirmed (A)	
LABBENZ	Negative	

[redacted]

No results found.

Lines/Drains/Airways
Line Duration

Peripheral I\[redacted] Anterior;Proximal;Right Forearm 2 days
Peripheral I\[redacted] Left Antecubital 2 days

Foley: N/A
Central line:N/A
DVT Prophylaxis: Mechanical SCDs

Allergy:Allergies
No Known Allergies
Home Medications:Medications Ordered Prior to Encounter<u>**Medications on current admission:**</u>
No current facility-administered medications on file prior to encounter.

No current outpatient medications on file prior to encounter.
Scheduled Medications:

• acetaminophen	650 mg	Oral	Q6H
• enoxaparin	30 mg	Subcutaneous	BID
• folic acid	1 mg	Oral	Daily
• levETIRAcetam	1,000 mg	Oral	BID
• magnesium sulfate	4 g	Intravenous	Once
• multivitamin	1 tablet	Oral	Daily
• polyethylene glycol	17 g	Oral	Daily
• senna-docusate	1 tablet	Oral	BID
• tamsulosin	0.4 mg	Oral	Daily
• thiamine	100 mg	Oral	Daily

• sodium chloride[redacted]
PRN Medications:sodium chloride[redacted] dextrose **OR** dextrose, insulin lispro

<u>**ASSESSMENT & PLAN:**</u>[redacted]57 y.o. male admitted o[redacted]No problem-specific Assessment & Plan notes found for this encounter.

Primary Diagnosis:
possibility of DAI
bifrontal subarachnoid hemorrhage
Mildly displaced comminuted bilateral nasal bone fractures
Minimally displaced left orbital floor fracture
Nondisplaced left inferolateral orbital rim fracture extending along the zygomatic arch
right superior orbital wall fracture

Neuro:
possibility of DAI
bifrontal subarachnoid hemorrhage
57 y.o.
GCS @ Admission Glasgow Coma Scale Score: 15
Most Recent Documented **GCS**: Glasgow Coma Scale Score: 15
- **precautions:** minimize benzos, anticholinergics, and opioids, Reduce <u>intrinsic</u> deliriogenic factors, Maintain nutrition. Reduce <u>extrinsic</u> deliriogenic factors: including sleep-wake cycles, sleep hygiene, reassurance and re-orientation, try to avoid restraints
- Restraints: I personally examined the patient and restraints are not indicated as per my clinical assessment today.
- multimodal pain control

[redacted]

- No emergent neurosurgical intervention
- Repeat CTH 6 hours from initial scan
- HOB >30 degrees
- PLT >100, INR [redacted]
- Na goal [redacted]
- Correct any Underlying Coagulopathy
    - [redacted] consider ROTEM to aid in identifying clotting abnormality
    - Reverse any AP/AC agents per protocol
- Initiate Prophylactic Keppra 500mg BID x7 days [redacted]
    - If evidence of seizures, [redacted] require further uptitration and further Neurology consultation
- Hold on any further surgical intervention until stability imaging obtained

Cardiac: Heart Rate: 7[redacted]lood Pressure: 100[redacted]
-
Respiratory: SpO2: 96 %[redacted]
- Incentive spirometry: use ten times per hour while awake.

GI:

Recent Labs

Lab [redacted]	
TOTALBILI	[redacted]
AST	22
ALT	18
ALKPHOS	80

No results found for: "MICROALBUR", "TOTALPRO", "TRANSTHY"

Lab Results

Component	Value	Date/Time [redacted]
ALBUMIN	[redacted]	

Body mass index is [redacted] kg/m².
- Diet: Regular Diet (ADULT)
Malnutrition Diagnosis:
- Bowel regimen: Miralax, pericolace
-
GU:

Recent Labs

Lab [redacted]			
MG	–	0.9	[redacted]
PHOS	–	[redacted]	1.4
NA	137	136	138
K	3.9	4.5	4.6
BUN	9	7	10
CREATININE	0.8	0.8	[redacted]

- appropriate IVF management
- replace potasium, magnesium, phosphate

Musculoskeletal:
Mildly displaced comminuted bilateral nasal bone fractures
Minimally displaced left orbital floor fracture
Nondisplaced left inferolateral orbital rim fracture extending along the zygomatic arch
right superior orbital wall fracture
- Mobility Goal for Today:ROM
- OMFS:
    - Ophthalmology consult for evaluation of L orbital trauma
    - Apply bacitracin to abrasions TID
    - Oral hygiene cares
    - **Sinus Precautions**
        - It is important that you avoid all of the following:
            - Blowing your nose - It is best to wipe away nasal secretions carefully. After 2

weeks, if you must blow your nose, blow gently through both sides at the same time. Do not pinch your nose; do not blow just one side at a time.
- Sneezing - If you must sneeze, keep your mouth open and do not pinch your nose closed.
- Sucking - Do not drink through a straw. Do not smoke.
- Blowing - Do not play a wind instrument. Do not blow up balloons.
- Pushing or lifting - Do not lift or push objects weighing more than 20 pounds.
- Bending over - Keep your head above the level of your heart. Sleep with your head slightly raised

- Ophthalmology:

No acute intervention per ophthalmology
- Defer further management of orbital fracture repair to facial trauma service
- Start ice to periorbital swelling for 20 min at a time every [redacted] hours for first 48 hours
- No nose blowing
- Patient should have outpatient ophthalmology follow up, on call resident to arrange.
- Review RD signs and symptoms with patient: [redacted] to ER for flashing lights, shower of floaters, or dark veil over vision
- RD precautions: no heavy lifting, straining, or bending over for 1 week

Endocrine:

Lab Results

Component	Value	Date
HGBA1C	[redacted]	[redacted]

Recent Labs

Lab	[redacted]								
GLUCOSE	89	90	95	118	100	95	102	84	102

- appropriate POC blood sugar monitoring

Heme:

Recent Labs

Lab	[redacted]		
HGB	[redacted]	[redacted]	[redacted]
PLT	73*	463*	480*

- Monitor Hgb
- transfuse if <7 and/or unstable
-

ID: [redacted] 15)Blood Pressure: 1[redacted]0815)Temp: [redacted] °F [redacted] °C [redacted]

Recent Labs

Lab	[redacted]		
WBC	[redacted]	[redacted]	[redacted]
LACTATE	[redacted]	–	–

- Antibiotics: N/A
- Most Recent Culture/Sensitivities:

[redacted]**lan:**

- Multimodal pain control
-BAIT/SUN
-SW for homelessness
-PT/OT
-ST for cognition
-Resiliency clinic post TBI for DAI

Consults: Ophtho, NSGY, OMFS
Dispo: Pending

[redacted]

Assessment and plan discussed and formulated with the cosigning attending physician [redacted]

Report Electronically Signed By [redacted]

Acute Care Surgery

[redacted]

[redacted]

ILLUMINATION FOUNDATION
RECUPERATIVE CARE

Inland Empire REFERRAL FORM

To refer a patient, please complete the entire form including supporting documents and **FAX** to [redacted] or **email** to **RECUP@ifhomeless.org** For all referral questions, please contact IF Referral Coordinator at **(951) 708-6000**

Referring Hospital [redacted] **Dept/Floo** [redacted]
SW/CM/RN [redacted] **Phone/pager#** [redacted]
E-mail: [redacted] **Nursing stati** [redacted]
Authorized by: [redacted] **phone#:** [redacted]

Patient Nam [redacted] **MR** [redacted] **Date of Birt** [redacted]

Gender: ☒ Male ☐ Female ☐ Transgender **Ethnicity:** Mexican **Social Security# (last 4):** xxx-xx-

☐ **ED Visit** ☒ **Hospital Admit** **Da** [redacted] **Expected Discharge Dat** [redacted]

Insurance Typ [redacted] **English Speaking?** ☒ Yes ☐ No ***Primary Language Spoken:** _____

PLEASE VERIFY: ☒ **Currently homeless** **Check if:** ☐ **Homeless for more than 1 year continuously or 4 episodes in last 3 years**

CHIEF COMPLAINT/ADMITTING DIAGNOSIS: Subarachnoid hemorrhage

SUBSTANCE AB ☐ None ☒ Alcohol ☐ Cocaine ☐ Heroin ☒ Meth ☒ Other: PCP
Last Date Used [redacted] Current withdrawals: ☒ No ☐ Yes *Please explain_____

ANY WOUNDS? ☐ Yes ☒ No #/Loc/Size/Stage:_____

Independent with wound care? ☐ Yes ☐ No If no, will Home Health be ordered? ☐ Yes ☒ no

PPD/TB test or Chest XRAY performed? ☐ yes ☒ no Date:_____ Results:_____

ANY LIMITATIONS, BEHAVIORAL CHALLENGES OR MENTAL HEALTH ISSUES? ☒ None ☐ Auditory/Visual Hallucinations

☐ Mental Health DX If yes, **what type?**:_____ ☐ Non compliant ☐ forgetful

☐ Cognitive Impairment ☐ Reg. Sex offender ☐ other, please explain _____

Requires O2? ☐ Yes ☒ No Self Administer All Meds? ☒ Yes ☐ No *Please explain:_____

Continent of Bowel & Bladder? ☒ Yes ☐ No Colostomy/Ileostomy ☐ Yes ☒ No Foley Catheter ☐ Yes ☒ No

Independent w/all ADL's? ☒ Yes ☐ No *Please explain:_____

Diabetic? ☒ No if ☐ Yes, then: ☐ Insulin ☐ Oral Meds Communicable Diseases?: N/A

Anticoagulants?: ☒ No IF ☐ yes, Requires INR/PT/PTT checks through Home Health or Clinic? _____

Ambulatory? ☒ Yes ☐ No Assistive device? ☒ No ☐ Yes *Please Explain:_____

Does Patient have a car? ☐ Yes ☒ No Spouse/Partner? ☐ Yes ☒ No Service Animal? ☐ Yes ☒ No

Length of Stay authorized in Recuperative Care:____days

Attached Docs: ☒ **Face Sheet** ☒ **H&P** ☒ **Med List** ☐ **Psych notes** ☒ **Surgical notes** ☐ **PT/OT Eval.** ☐ **SW notes**

IF STAFF ONLY

Date Received:_____ Reviewed by:_____ Approved: ☐ Yes ☐ No Date:_____ Auth#_____

Denied Reason?:_____ Admission Date/time:_____ Exit Date:_____

Extension Requested?: ☐ no ☐ yes If yes, Approved additional days?:_____ by:_____ TOTAL LOS:_____

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